



HEXAMED

DIAGNOSTICS & SPECIALTY CLINICS

Your Right to Right Healthcare...



Barcode No **89781072**
Patient Name **Mrs.SHANAZ AKHTER**
Age/Sex **33 YRS/Female**
Referred By **DR. SUNIL K CHOWDHARY**

Lab No **16952501090004**
Reg Date **09/Jan/2025 12:54PM**
Sample Coll. Date **09/Jan/2025 11:56 AM**
Sample Rec.Date **09/Jan/2025 01:06 PM**
Report Date **11/Jan/2025 11:43AM**

IMMUNOLOGY SPE

Test Name With Methodology	Result	Unit	Biological Ref.Interval
Quadruple Test			
AFP. Chemiluminescent immunoassay (CLIA)	74.33	ng/ml	
uE3 Chemiluminescent immunoassay (CLIA)	2.850	ng/mL	
Beta Hcg Titer Chemiluminescent immunoassay (CLIA)	19332	mIU/mL	
Inhibin A. Chemiluminescent immunoassay	326.6	pg/ml	

Comments:

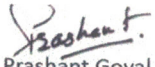
Statistical evaluation of Quadruple marker has been done using **Benetech PRA** software.

The test is valid between 15 Weeks to 21 weeks & 6 days of gestation. (optimally performed between 16 and 18 weeks). Quadruple marker test in Second trimester, identifies pregnancies that have a high risk of babies having the following chromosomal abnormalities:

1. NTD (Neural tube defect)
2. Trisomy 21 (Downs syndrome)
3. Trisomy 18 (Edwards syndrome)

Quadruple screening test includes testing for alpha-fetoprotein (AFP), human chorionic gonadotrophin (hCG), unconjugated estriol (uE3), and inhibin A. High AFP levels may indicate that the baby has an open neural tube defect. Lower than normal AFP levels could indicate that a woman is at higher risk for having a baby with Down syndrome. Levels of hCG and Inhibin-A are higher than normal when a woman has an increased risk of having a baby with Down syndrome. Lower than normal levels of Estriol may also indicate that a woman is at high risk for having a baby with Down syndrome.

This Screening test is NOT a diagnostic test (it cannot tell whether the baby has any of the above conditions); it only identifies the risk of the baby being affected. A confirmatory test may be needed when the screening test is positive (high risk). About 5 percent


Dr. Prashant Goyal (DCP)
(Director & Chief Pathologist)
Reg. No. DMC-53016

A Unit of Hexamed Healthcare Services LLP

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QUADRUPLER MARKER

Patient Name: Mrs. SHANAZ AKHTER
Race: INDIAN
Physician:

Code: 89781072
DOB: 01/01/91
Reported: 11/01/25

CLINICAL INFORMATION

Estimation Method: by U/S (21 wks 3 days on 07/01/25)
Age at Term: 43.5 years
EDD: 17/05/25
Gestation: Singleton
Maternal History: IDDM(U), SMOKER(U), IVF(Y)
Donor DOB: 16/11/81
Gestational Age: 21 weeks 5 days
Referring Lab #: 89781072
Specimen Code: 89781072
Specimen Date: 09/01/25
Received Date: 09/01/25
Weight: 79.0 kg
Screening Status: Initial sample
Para / Gravida: 0 / 0

REMARKS

Down Syndrome

The risk of Down syndrome is LESS than the screening cut-off. The serum screen has indicated a REDUCED risk from that based on maternal age. No follow-up is indicated regarding this result.

NTD

The maternal serum AFP result is NOT elevated for a pregnancy of this gestational age. The risk of an open neural tube defect is less than the screening cut-off.

Trisomy 18

These serum marker levels are not consistent with the pattern seen in Trisomy 18 pregnancies. Maternal serum screening will detect approximately 60% of Trisomy 18 pregnancies.

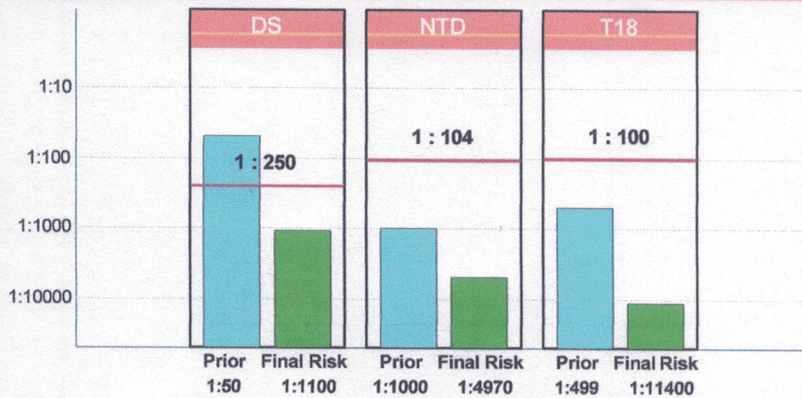
BIOCHEMISTRY

MARKER	RESULT	MoM
AFP	74.3 ng/mL	1.14
uE3	2.85 ng/mL	1.14
hCG	19332.0 mIU/mL	0.92
Inhibin A	326.6 pg/ml	1.38

INTERPRETATION

Down Syndrome: Screen Negative
NTD: Screen Negative
Trisomy 18: Screen Negative

CLINICAL RESULTS (at term)



The calculated risk by Benetech depends on the accuracy of the information Provided by the referring physician. Please note that calculation are statistical approaches and have no diagnostic value.

Report Validated by DR
 PRASHANT GOYAL

Report generated using Benetech PRA 3403, Reprinted on 11/01/25



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of women have a false-positive result, meaning that the test result is positive but the baby does not actually have Down syndrome.

The interpretive unit is MoM (Multiples of Median) which takes into account variables such as gestational age (ultrasound), maternal weight, race, insulin dependent Diabetes, multiple gestation, IVF (Date of Birth of Donor, if applicable), smoking & previous history of Down syndrome. Accurate availability of this data for Risk Calculation is critical.

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